

DEPRESSION

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DEPRESSION IN SINGAPORE

Major depression is the most common mental illness in Singapore. Based on National Mental Health Study in 2016, 6.2% of the adult population suffered from major depression at some point in their lifetime.¹

Among all people with a mental illness, 22.1% had consulted a psychiatrist, and 18% went to a GP. 14.3% of people with a chronic illness had a mental illness, and 50.6% of people with a mental illness also had a chronic illness.²

Diabetic patients have increased depressive symptoms.³ Depression also worsens the prognosis for diabetes. It increases non-compliance to medical treatment and decreases the quality of life.

The treatment gap for depression was high. The 12 months treatment gap for Major Depressive Disorder was 73%. 45.6% met the criteria for moderate disorders.⁴ Those highly educated and employed were less likely to seek help than those of lower education. Those with chronic physical conditions were less likely to seek treatment as well.

SCREENING FOR DEPRESSION

Opportunistic screening for depression is not recommended because of limited evidence based on its effectiveness. However, patients with risk factors should be screened².

Risk Factors for Depression Screening

Clinical Factors	Symptoms Factors
• History of depression • Family history of depression • High users of the medical services & presence of chronic medical conditions (especially cardiovascular disease, diabetes, and neurological disorders) • Other psychiatric conditions • Times of hormonal challenge (e.g., peripartum)	• Unexplained physical symptoms • Chronic pain • Fatigue • Insomnia • Anxiety • Substance abuse

Depression Screening can be done with PHQ-2

Over the last 2 weeks, how often have you been bothered by any of the following problems:

1. Little or pleasure in doing things.
2. Feeling down, depressed, or hopeless.

If the PHQ-2 score is 3 or more, patient should be assessed with a PHQ-9. Patients with PHQ-9 score of 10 or more should be evaluated by a doctor. Evaluation includes history taking, mental state examination, physical examination, and necessary investigations to determine if the patient has a mental disorder, and to exclude biological causes.

SUICIDE RISK ASSESSMENT

Suicide assessment refers to an evaluation to identify warning signs, risks, and protective factors to estimate the immediate danger to the patient and decide on the course of therapy. Patients reporting a recent suicide attempt or experiencing suicidal ideation may present themselves to any staff. Patients with these complaints should receive a suicide risk assessment by a HMC-trained care manager, doctor or psychologist on the same day. Examples of patients with higher risks of attempting suicide include but are not limited to:

- a) Attempted self-harm; threatens to hurt or kills self or looking for ways to kill self (seeking pills or other means),
- b) Dramatic changes in mood or reckless actions, not speaking coherently,
- c) Talking about death, suicide or making plans to divest responsibility or financial arrangements,
- d) Expressing hopelessness, no reason for living or no purpose in life, or
- e) Withdrawal from friends, family, or society.

Standardised assessment tools such as **C-SSRS (nationally preferred scale) or P4** can be used to support this process. This should not replace clinical assessment. Patients with low or medium suicide risk but deemed safe for home should be given appropriate follow-up appointments. Patients with high suicide risk should be transferred to the hospital. Please refer to [NUP-WI-CS-COP-032 Management of Suicidal Patients](#) for further guidance. **Suicide Safety Plan can be retrieved using the communications tab in EPIC by using .NUPSAFETYPLAN SmartPhrase to aid your consultation.**

DSM V CRITERIA FOR DEPRESSION

- 5 or more of the following 9 symptoms, at least one criterion involving (1) or (2)
 1. Depressed mood (feels sad, empty hopeless), persistent for most of the time, for 2 weeks duration (Note: In children and adolescents, can be irritable mood.)
 2. Reduced in interest or pleasure in almost all activities, most of the day, for 2 weeks duration
 3. Weight gain or loss; increase or decrease of appetite
 4. Insomnia or hypersomnia
 5. Psychomotor agitation or retardation
 6. Fatigue or loss of energy
 7. Feelings of worthlessness or inappropriate guilt
 8. Poor concentration or indecisiveness
 9. Thought of death or suicidal ideation
- Significant distress or functional impairment in social and or occupational areas.
- Never had a manic or hypomanic episode.

DIFFERENTIALS FOR DEPRESSION

Medical Conditions Which Can Cause Depressive Symptoms

1. Endocrine – Hypothyroidism, Cushing disease, Addison disease
2. Malignancy
3. Neurological conditions – Stroke, Syphilis, Tumour, Parkinson's disease
4. Chronic illness – Heart Failure, Systemic Lupus Erythematosus
5. Sleep disorders

Psychiatric Conditions to Consider

1. Psychotic Disorders, i.e., Schizophrenia, Schizoaffective Disorder, Delusional Disorder
2. Bipolar Disorder (may present first as unipolar depression)
3. Comorbid anxiety Disorder, Post Traumatic Stress Disorder

Drugs That May Cause or Aggravate Depressive Symptoms or Cause Drug Interactions with SSRI's

Chronic Conditions	Beta blockers, Statins i.e., simvastatin
Endocrine / Hormones	Prednisolone, progestogens, oestrogen
Neurology	Levodopa, Bromocriptine, Anticonvulsants, i.e., Gabapentin, Topiramate
Others	Proton Pump Inhibitors, Ciprofloxacin
Substance Abuse	Alcohol, benzodiazepines, opioids

MANAGEMENT OF DEPRESSION

Management of mental illness is usually thru the bio-psycho-social-spiritual model. A case formation looking at the predisposing, precipitating, perpetuating factors, and protective factors is often a useful guide to devise the management strategy.

	Predisposing	Precipitating	Perpetuating	Protective
Biological	Genetic, Family history	Medical issues i.e., pain	Chronic medical issues i.e., pain	Biological treatment available
Psychological	Childhood trauma, negative cognitions	Acute grief	Pessimistic personality	Help seeking
Social	Exposure to domestic violence	Financial crisis	Financial difficulties	Help seeking, social connectedness, meaningful relationship(s) and activity(ies)

PHQ-9 Interpretation and TCU / Referral Recommendations

*The severity of depression is based on function, clinical impression, and the risk of harm, aided by scales

Suicide Risk	PHQ-9 Score	Severity	TCU/Referral Recommendation
No / Low suicide risk	0 - 4	Minimal	Reassure patient, routine follow up, encourage regular exercise and self-care
	5 - 9	Mild	- Psychological education + simple self-help strategies - TCU MSW (Reason for referral: Treatment related assistance- psychosocial support) for cognitive behavioural therapy (CBT) within 8 weeks
	10 - 14	Moderate	- TCU Psychologist (Short) (First Visit) if agreeable within 4 weeks - TCU Dr HMC Long (FV) for consideration of medication within 4 weeks
	15 - 19	Moderately severe	- TCU Psychologist (Short) (First Visit) if agreeable within 2 weeks - TCU Dr HMC Long (FV) for consideration of medication within 2 weeks; you may start SSRI's first if you assess that it is appropriate and patient is agreeable
	20 - 27	Severe	
Moderate suicide risk	Any	Any	- Refer SOC (Direct Access) - Safety planning (.nupsafetyplan)
Severe suicide risk	Any	Any	Refer ED

1. Pharmacotherapy

Antidepressants are effective for moderate to severe depression. It is also effective for anxiety and obsessions. Its onset of action is usually around 3 - 4 weeks. Onset of side effects are immediate and get better within 2 weeks or so.

2. Psychotherapy

Psychotherapy is effective for mild to moderate depression. Evidence supports concurrent use of pharmacotherapy and psychotherapy, which yields better improvement and a lower relapse rate. The most common evidence-based intervention for depression is cognitive behavioural therapy (CBT). Other evidence-based therapy includes interpersonal psychotherapy and mindfulness based cognitive therapy. Patients who are not suitable for therapy include not being motivated, highly suicidal, highly symptomatic, psychotic etc.

3. Social issues

Patients with social issues, including financial, family and relationship difficulties, can be referred to NUP medical social workers and financial counsellors. Family service centres provide counselling for social issues, including marital discord, divorce, parenting issues, grief, domestic violence *etc.*

Pharmacotherapy

Many anti-depressants are affected by pharmacogenomics. Please read the EPIC banners that pop up whilst prescribing these medications, as the warnings are nuanced, extensive, and will not be able to be covered in CPG's. If in doubt, clarify with clinic pharmacists.

Antidepressants in NUP

Class of Drug	Name of Drug	Dose	Side Effects	Remarks
SSRI (Selective Serotonin Reuptake Inhibitor)	Fluoxetine (S2) (10 mg and 20 mg capsules) \$\$^A	Initiation 10-20 mg every morning Maintenance 20-80 mg	Common side effects: Headache, GI (nausea / diarrhoea), excessive daytime somnolence, orthostatic hypotension, insomnia , anticholinergic side effects. Most of the side effects will improve with time. Uncommon side effects: Sexual dysfunction, Tremors, akathisia, QTc prolongation, SIADH, hyponatremia (elderly at increased risk), bleeding risk (especially with concurrent use of antiplatelets / anticoagulants) Serotonin syndrome, i.e., myoclonus, hyperreflexia – especially when in combination with TCAs, MAOI etc.	Fluoxetine is more likely to cause insomnia.
	Fluvoxamine (S2) (50 mg tablets) \$\$^A	Initiation 25-50 mg every night Maintenance 50-200 mg Doses > 100 mg/day in 2 divided doses		Generally considered safe in renal and hepatic impairment. Use low dose and titrate slowly.
	Sertraline (S2) (50 mg tablets) \$\$^A	Initiation 25-50 mg every morning Maintenance 50-200 mg		Avoid fluoxetine in hepatic impairment due to extensive hepatic metabolism and long half-life.
	Escitalopram (NS) (10 mg tablets) \$\$\$\$^A	Initiation 5-10 mg every morning Maintenance 10-20 mg		Sertraline preferred SSRI for pregnancy and cardiac diseases. Escitalopram has less drug-drug interactions, but may cause weight gain

Class of Drug	Name of Drug	Dose	Side Effects	Remarks
Considerations for SSRI - Often first line treatment due to safer side effect profile - Avoid in patients with bipolar disorder or those suspected to have bipolar disorder (4 or more days of symptoms suggestive of hypomania) - Consider doing a baseline ECG, ensuring that QTc is below 500ms, especially when starting escitalopram or trazodone - Contraindicated in patients on monoamine oxidase inhibitor (MAOI) - For patients with history of acute angle-closure glaucoma to have a risk benefit discussion with patients, and to write a memo to attending ophthalmologist for discussion and monitoring of eye pressure if started on SSRI's. There is conflicting evidence as to whether SSRI's affect intraocular pressure if at all.				
SNRI (Selective Serotonin and Norepinephrine Reuptake Inhibitor)	Venlafaxine XR (S2) (75 mg capsules) \$\$^	Initiation 75 mg every morning Maintenance 150-225 mg	Common side effects: Headache, sweating, nausea, dry mouth, constipation, nervousness, insomnia, Dose dependent increase of blood pressure Serious side effects: Serotonin syndrome, SIADH/hyponatremia (predominantly in the elderly), sexual dysfunction	Not recommended in patients with angle closure glaucoma, seizures. Use in caution with hepatic and renal impairment. Dosage reduction recommended.

Class of Drug	Name of Drug	Dose	Side Effects	Remarks
NaSSA (Noradrenergic and specific serotonergic antidepressant)	Mirtazapine (S2) (15 mg tablets) \$\$^A	Initiation 7.5-15 mg every night Maintenance 15-45 mg	Common side effects: Sedation, weight gain, appetite gain, Dry mouth and constipation	Low doses of mirtazapine are often used for patients with concomitant insomnia, as it preferentially blocks the histamine receptor. At higher doses, the antihistamine activity is offset by increased noradrenergic transmission, which reduces its sedating effect. Use of higher doses at bedtime may not be advisable and may worsen certain neuropsychiatric symptoms. There is less sexual and nausea side effect, and less hyponatremia compared to SSRI. Use with caution in patients with hepatic and renal impairment.

Class of Drug	Name of Drug	Dose	Side Effects	Remarks
TCA (Tricyclic Antidepressants)	Amitriptyline (S1) (10 mg and 25mg tablets) \$^	Initiation 10-25 mg every night Maintenance 50-100 mg	Common side effects: Anticholinergic side effects: Dry mouth, constipation, blurred vision, urinary retention, weight gain, Dizziness, somnolence, drowsiness / headache, Fatigue Intraocular pressure, Palpitations, tachycardia, orthostatic hypotension	Good for headache or insomnia as well
	Nortriptyline (10 mg and 25 mg capsules) (Not available in NUP)	Initiation 10-25 mg every night Maintenance 50-150 mg		Less anticholinergic side effect
Considerations for TCA • Generally not used as first line therapy due to increased anticholinergic and cardiotoxic side effects • Contraindicated with patients on monoamine oxidase inhibitor (MAOI) • May lower seizure threshold • Use with caution in patients with hepatic or renal dysfunction and in elderly patients • Toxic cardiac effects if overdose • Not to be used in patient during acute recovery phase following myocardial infarction				

^ Cost

\$ < \$10 per month

\$\$\$\$ \$30 - <\$60 per month

\$\$ < \$10 - < \$20 per month

\$\$\$\$\$ \$60 - <\$90 per month

\$\$\$ \$20 - < \$30 per month

\$\$\$\$\$\$ \$90 or more per month

Amount payable depends on patient subsidy level and drug subsidy class (Standard Drug List S1 and S2, Non-Standard Drug NS)

Using Antidepressants

Initiation⁴

- Selective serotonin reuptake inhibitor (SSRI) is the class of choice for initial therapy due to its effectiveness, tolerability, and safety in overdose. It is the drug of choice for depressed persons with cardiovascular disease.
- If there is a **partial response after 4 weeks, consider increasing** dosage for better response.
- **If there is no response after 4 to 8 weeks, consider switching** to another SSRI, before switching to another class.
- **There is a possible risk of suicidal behaviour and self-harm during the initial 1 to 3 months of commencing antidepressants. Do order a CM HMC Teleconsultation at around 2 weeks after starting SSRI's (or other classes) for nurses to check on tolerability, adherence to medications, perform suicide risk assessment, and look for early conversion to mania/hypomania.**
- Can reduce dosage if there are tremors or akathisia.
- Stop antidepressant if there are hypomanic / manic symptoms, i.e., elevated mood or irritability, reduced need for sleep, increased energy, overfamiliar, grandiose, psychotic. Refer to psychiatrists direct access to manage potential bipolar disorder.
- For patients drinking alcohol, warn them that alcohol is itself a depressant and that drinking alcohol can worsen their symptoms. It is better to avoid alcohol whilst on antidepressants. Chronic alcohol use also acts as an enzyme inducer which then reduces effectiveness of certain antidepressants.

Stopping Antidepressant⁵

- Patients should be advised on the risk of relapse and discontinuation syndrome if medications are abruptly discontinued.
- Stopping antidepressants is usually considered for patients with the 1st depressive episode who have responded to medications for at least 6 to 9 months after remission of symptoms, the precipitating factor for depression has resolved, and the patient feels ready to reduce and stop medications.
- Antidepressants are usually tailed down over 4 weeks or more depending on patient's response.
- Discontinuation symptoms are flu-like symptoms, sleep disturbance, GI upset, diaphoresis, irritability, mild anxiety, increase dreaming, ataxia, paraesthesia, tremors of fingers and fatigue.
- **Warn patients about the potential risk of self-harm being the highest 1 month after stopping an antidepressant and to seek help should it occur.**
- Follow up on the patient for at least 6 months after stopping antidepressants, as the risks of recurrence are highest during this time. The follow up can be done via virtual consult if patient prefers.
- If the patient relapses, he is likely to respond to the same antidepressant at the same effective dosage.

Consider long term maintenance treatment in patients with severe depressive episodes (suicide tendencies, psychotic features, and severe functional impairment) and previous multiple depressive episodes. Patients with ≥ 3 episodes of depression should be on long term antidepressants.

Use of Antidepressants in Young Adults (18-24 years old)

- Young adults (18-24 years old) on antidepressants have to be closely monitored.
- Drug of choice: SSRI e.g., Fluoxetine, Escitalopram. Sertraline has been put in NICE guidelines as an alternative, although it is not yet FDA approved.
- Monitor for worsening suicidal ideation, unusual changes in behaviour during the first 1-2 months of the initiation phase.
- Educate patient on black box warning, suicidality risk and swing to hypomania / mania. Number needed to treat: 10, and number needed to harm: 143. Overall, benefits of treatment far outweigh risks of treatment.
- Ensure adequate psychosocial support and assess suicide risk at every visit until stabilisation.
- Communicate with parents or family members if present with the patient's consent.
- Consider referral to a psychiatrist if the patient develops side effects or is not responding to SSRI.
- Maintenance phase: Stable patients can be reviewed every 2-4 months.
- SSRI and other antidepressants are related to a higher risk of suicidal behaviour in children, adolescents, and young adults but no complete suicides were reported in the trials. Overall, the potential benefits of treatment with antidepressants outweigh the risks in relation to the suicidal behaviour.^{7,8} The use of antidepressants in children and young adults under the age of 18 should include close monitoring for suicidal thoughts and behaviour.

Use of Antidepressants in older adults (above 65 years old)

- Psychotherapy remains the preferred treatment for the elderly if receptive and feasible.
- Considering monitoring sodium while the elderly is on antidepressants (especially SSRIs and SNRIs).

Serotonin Syndrome⁶

- Serotonin syndrome is an Adverse Drug Reaction, which can be life threatening. It usually occurs with combination therapy, e.g., SSRIs, SNRIs, MAOIs, TCAs, valproate, antiemetics, tramadol, dextromethorphan. Serotonin syndrome is a diagnosis of exclusion. No single diagnostic test can confirm this syndrome.
- Patients present with a triad of symptoms of altered mental state, autonomic symptoms & neuromuscular excitation.
- In mild cases, the predominating features are mild hypertension and tachycardia, mydriasis, diaphoresis, shivering, tremor, myoclonus, and hyperreflexia. Patients with mild serotonin syndrome are usually afebrile.

- Patients with moderate serotonin syndrome usually have the above symptoms plus hyperthermia (40°C), hyperactive bowel sounds, horizontal ocular clonus, mild agitation, hypervigilance, and pressured speech.
- In severe cases, patients have all the above symptoms plus hyperthermia greater than 41.1°C, dramatic swings in pulse rate and blood pressure, delirium, and muscle rigidity. Severe cases may result in seizures, rhabdomyolysis, myoglobinuria, metabolic acidosis, renal failure, acute respiratory distress syndrome, respiratory failure, diffuse intravascular clotting, coma, and death.
- Management would be to discontinue the offending agent and refer patient to the A&E for monitoring and support.

Role of Health Team Members

1. Care Manager (CM)

- Conduct depression / anxiety screening using PHQ-2 / GAD-2 where relevant and proceed to PHQ-9 / GAD-7 if PHQ-2 / GAD-7 scores are ≥ 3 respectively.

2. Family Physician (Dr)

- Refer patients with psychological problems who fulfil inclusion criteria to Health and Mind Clinic (HMC) in NUP (those aged ≥ 18 years with mild-moderate depression, anxiety, adjustment disorders, insomnia).
- Manage stable patients discharged from HMC.
- Refer to Psychiatry SOC based on geographical boundaries for those with high risk of suicide or risk of harm to self or others, or patients who are part of the exclusion list of HMC e.g., addictions / legal issues / bipolar disorder / new onset psychosis / new onset OCD.
- Start patients with moderate-severe depression on appropriate SSRI's (consult senior doctors if unsure) and give an appointment with HMC in 2 weeks' time. If there are no available HMC slots, it would be prudent to schedule a doctor review in teamlet / general pool to check on the patient.

3. Care Manager in Health and Mind Clinic (HMC CM)

- Conduct depression, anxiety, insomnia, and suicide risk screening for patients visiting HMC Dr for the first time.
- Offer psychoeducation and basic self-help techniques,
- Details to refer to NUP-WI-CS-COP-030 Management of Patients in Health and Mind Clinic.

4. Family Physician in Health and Mind Clinic (HMC Dr)

- Manage new cases and follow-up cases of depression / anxiety / insomnia / adjustment disorders who are 18 years old and

above.

- Follow-up psychiatric stepdown cases that are not suitable for teamlet care.
- Escalate patients to SOC if needed.
- Details to refer to NUP-WI-CS-COP-030 Management of Patients in Health and Mind Clinic.

5. Psychologist

- Conduct psychological screening and assessment during initial assessment to assess the severity of mental health symptoms, complexity of issues and presence / absence of risk tendencies and behaviours.
- Formulate treatment plan for follow-up visits.
- Can work with many types of cases inclusive grief; full list as per [Allied Health - Psych Service](#).

6. Medical Social Worker

- Care assessment and arrangement, **supporting mild cases of depression, anxiety, or adjustment disorder with cognitive behavioural strategies, dealing with mild insomnia**, supportive counselling, crisis intervention, non-medical related financial assistance, and support patients in need of information and referral.

7. Financial Counsellor

- Patients with financial difficulties on medical bills could be referred to Financial Counsellor for assistance.

REFERRAL TO SPECIALIST

- Patients 17 years and below who might need antidepressants.
- If patient has not responded to 3 different antidepressants.
- Presence of complicated medical history or special circumstances, e.g., in an antenatal state or postpartum within 1 year (refer KKH or NUH WEHS), breastfeeding, Cushing disease, liver disease *etc.*
- Presence of comorbid personality disorders, substance dependence.
- New onset psychotic disorder or bipolar disorder.
- **Potential** need for **interventional psychiatry, e.g.,** ECT (electroconvulsive therapy), neurostimulation, i.e., TDCS (transcranial direct current stimulation), rTMS (repetitive transcranial magnetic stimulation), *etc.* *These services would be referred to by the treating psychiatrist in hospital if required*

STEP-DOWN CARE

NUP accepts step down care with the following conditions:

- Patients with mental diagnoses that are predominantly depression or anxiety. They are stable and mild to moderate in severity. They may be seen within the teamlet construct if **very** stable. If they require titration of medications, or still are quite symptomatic, **or risks relapse**, they are to be kept within HMC. There is no need to refer these patients back to the hospital unless they are **moderate to high risk**.
- Patients should be on medications in the NUP formulary. Patients should not be reliant on regular benzodiazepines. Some patients (anxiety disorders) may require occasional benzodiazepines. Supplying **up to 5** tablets of benzodiazepines in addition to 3 months of chronic medications is acceptable. Stand-alone top ups of benzodiazepines are discouraged.
- Some step-down cases are treated with two antidepressants. Such patients have an increased risk of serotonin syndrome. Some step-down patients may be on antipsychotics for augmentation. For those on antipsychotics, do BMI, fasting glucose, lipid, BP check every year.

RECOMMENDED CARE COMPONENTS FOR MAJOR DEPRESSION

Recommended Care Components	Minimum Frequency*	Remarks
Doctor review	Twice a year or longer if stable	Consultation includes assessment for symptoms, response and adherence to medications, psychosocial interventions, risk of harm to self or others, general physical health, and basic emotional support
PHQ-9 Score^	Every clinical review when appropriate, minimally 6 monthly for patients with depression	

^ These are reportable clinical indicators

** More frequently if clinically indicated*

POSTPARTUM DEPRESSION (PPD)

Please refer to the [NUP Women's Health CPG](#) for guidelines on antepartum and postpartum depression.

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